

The 5 Labs Every High-Performing Adult Should Track

What your annual physical leaves out, why each one matters, and how to get them at a fraction of retail cost

A free guide from LATOM Wellness

Why your annual physical isn't enough

Most physicals run the same panel: glucose, basic lipids, basic metabolic, a CBC, sometimes a TSH. That panel was designed to catch already-developed disease. It is not designed to find early dysfunction, to optimize, or to predict.

The five labs below catch things 10 to 20 years before standard care does. They are inexpensive when ordered directly through a clinical lab network like Fullscript. And they tell you what you actually need to know to optimize your health, not just avoid a hospital admission.

Read on. Then book a 30-minute consultation at latomwellness.com if you want help interpreting your numbers and building a personalized plan.

Lab #1: Fasting Insulin

WHAT IT TELLS YOU

Your body produces insulin to manage blood sugar. When cells start resisting insulin, your body produces MORE insulin to keep blood sugar normal. Insulin climbs for years (sometimes decades) before glucose itself starts to climb. Standard care catches the problem when glucose finally rises (your HbA1c crosses 5.7 percent for prediabetes). By then, you have been insulin resistant for 10 to 20 years.

Fasting insulin catches this much earlier. It is one of the highest-leverage labs in medicine.

WHAT STANDARD CARE TYPICALLY ORDERS

HbA1c and fasting glucose. Both late markers.

OPTIMAL RANGE

Below 7 microIU/mL is optimal. Most healthy-feeling adults walking around have fasting insulin between 10

and 20. That is silent metabolic dysfunction.

WHAT YOU CAN DO WITH THE NUMBER

- Below 7: your insulin sensitivity is in good shape
- 7 to 10: early signal, lifestyle interventions are highly effective
- 10 to 15: insulin resistance is present, supplement and lifestyle protocol indicated
- Above 15: significant insulin resistance, prescription consideration may be appropriate (GLP-1, metformin)

HOW TO GET IT

Order through Fullscript or your physician. Typical patient cost via direct laboratory networks: \$15 to \$30. You must fast 8 to 12 hours before the draw.

Lab #2: ApoB (Apolipoprotein B)

WHAT IT TELLS YOU

LDL cholesterol is the "bad cholesterol" everyone talks about. But LDL-C measures the WEIGHT of cholesterol in your LDL particles. ApoB measures the NUMBER of atherogenic particles in your blood. Each particle has one ApoB protein on it, regardless of how much cholesterol it carries.

Why does that matter? Because two patients with identical LDL-C can have very different cardiovascular risk depending on particle count. Small dense LDL particles (high count, low cholesterol per particle) are more atherogenic than large fluffy LDL particles (low count, high cholesterol per particle). ApoB captures this directly.

ApoB is now recognized in major cardiovascular guidelines as a better single marker for atherosclerotic risk than LDL-C alone.

WHAT STANDARD CARE TYPICALLY ORDERS

A standard lipid panel: total cholesterol, LDL-C, HDL-C, triglycerides. ApoB is rarely ordered.

OPTIMAL RANGE

- Below 80 mg/dL: optimal for primary prevention
- Below 60 mg/dL: target if you have established cardiovascular disease or strong family history
- 80 to 100: moderate risk, intervention worth considering
- Above 100: elevated risk, aggressive lifestyle plus consideration of medical therapy

WHAT YOU CAN DO WITH THE NUMBER

ApoB above 80 in a non-diabetic adult is a strong signal to:

- Reduce saturated fat intake
- Increase soluble fiber (psyllium, glucomannan, oats)

- Consider berberine, citrus bergamot, or red yeast rice supplements
- Discuss statin or ezetimibe with your physician if family history is strong

HOW TO GET IT

Order through Fullscript laboratory network. Typical patient cost: \$15 to \$35. No fasting required (unlike a standard lipid panel).

Lab #3: Lipoprotein(a) - Lp(a)

WHAT IT TELLS YOU

Lp(a) is a specific type of LDL particle with an extra protein (apolipoprotein(a)) attached. It is highly atherogenic AND highly thrombotic. About 1 in 5 adults has elevated Lp(a). Most do not know it.

The catch: Lp(a) is almost entirely genetically determined. Diet, exercise, and most medications barely move it. But knowing your Lp(a) changes how aggressively you should manage everything else (lipids, blood pressure, lifestyle).

If your Lp(a) is high, you have a built-in cardiovascular risk multiplier that most physicians have never tested for. If your Lp(a) is normal, you have meaningful reassurance even with mediocre lifestyle.

WHAT STANDARD CARE TYPICALLY ORDERS

Almost never. Lp(a) is not part of any routine cardiovascular workup despite major cardiology societies recommending it be measured at least once in every adult.

OPTIMAL RANGE

- Below 30 mg/dL (below 75 nmol/L): low risk
- 30 to 50 mg/dL (75 to 125 nmol/L): borderline elevated
- Above 50 mg/dL (above 125 nmol/L): elevated, requires aggressive risk factor management
- Above 100 mg/dL: very high risk, consider specialist consultation

WHAT YOU CAN DO WITH THE NUMBER

- Lp(a) high: lower your ApoB target aggressively, optimize blood pressure, quit nicotine if any, address every other modifiable risk factor. Discuss specialty therapies (PCSK9 inhibitors, emerging Lp(a)-specific medications) with cardiology.

- Lp(a) normal: meaningful reassurance. Standard cardiovascular optimization still indicated.

HOW TO GET IT

Order through Fullscript. **You only need to test Lp(a) ONCE in your adult life.** It does not meaningfully change. Typical patient cost: \$25 to \$50.

Lab #4: hsCRP (high-sensitivity C-reactive protein)

WHAT IT TELLS YOU

hsCRP is a marker of low-grade systemic inflammation. Chronic low-grade inflammation drives atherosclerosis, insulin resistance, neurodegeneration, and most chronic diseases of aging. Elevated hsCRP independently predicts cardiovascular events, cancer, and mortality, even in patients with normal cholesterol.

The "high-sensitivity" version of CRP (hsCRP) measures the very low levels relevant to chronic disease (not the acute-illness range that standard CRP catches).

WHAT STANDARD CARE TYPICALLY ORDERS

Sometimes standard CRP if rheumatologic or inflammatory disease is suspected. Almost never hsCRP for preventive screening.

OPTIMAL RANGE

- Below 1.0 mg/L: low cardiovascular risk
- 1.0 to 3.0 mg/L: average risk
- Above 3.0 mg/L: elevated risk
- Above 10 mg/L: acute or significant chronic inflammation, requires evaluation

WHAT YOU CAN DO WITH THE NUMBER

hsCRP above 1.0 should prompt:

- Investigation of potential drivers: dental infection, gut dysbiosis, sleep apnea, chronic stress, autoimmune signal, sedentary lifestyle, visceral adiposity
- Anti-inflammatory interventions: omega-3 fatty acids 2 to 4 grams daily, vitamin D repletion (target above 50 ng/mL), curcumin, quercetin
- Address the upstream causes

HOW TO GET IT

Order through Fullscript. Typical patient cost: \$10 to \$25. No fasting required.

Lab #5: Vitamin D 25-Hydroxy

WHAT IT TELLS YOU

Vitamin D is technically a hormone. It plays a role in nearly every tissue in the body: immune function, bone health, muscle strength, mood, glucose metabolism, fertility, sleep quality. Deficiency is extraordinarily common, especially in adults who work indoors, in northern climates, in patients with darker skin, and in anyone above age 50.

The right test is 25-hydroxy vitamin D (the storage form), not 1,25-dihydroxy vitamin D (which is the active form and is regulated separately).

WHAT STANDARD CARE TYPICALLY ORDERS

Sometimes. Often not, unless osteoporosis or other indication is present.

OPTIMAL RANGE

- Below 30 ng/mL: deficient
- 30 to 50 ng/mL: insufficient (most people in this range)
- 50 to 80 ng/mL: optimal
- Above 100 ng/mL: too high (very rare)

WHAT YOU CAN DO WITH THE NUMBER

- Below 30: aggressive repletion, 10,000 IU vitamin D3 daily for 8 weeks, then retest
- 30 to 50: maintenance dose 5,000 IU vitamin D3 daily
- Above 50: maintenance dose 2,000 to 3,000 IU vitamin D3 daily
- Pair with vitamin K2 (MK-7 form, 100 to 200 mcg) to ensure calcium routes to bone rather than vasculature

HOW TO GET IT

Order through Fullscript. Typical patient cost: \$20 to \$40.

What this looks like as a package

If you want all five labs together as a baseline workup:

Lab	Approximate cost
Fasting insulin	\$15 to \$30
ApoB	\$15 to \$35
Lp(a)	\$25 to \$50
hsCRP	\$10 to \$25
Vitamin D 25-OH	\$20 to \$40
Total bundle	**\$85 to \$180**

Compare to: ordering these individually through your insurance often results in much higher out-of-pocket costs after deductibles. Or you can simply not get them at all, which is what most patients end up doing.

The hard part is not getting the labs

The hard part is knowing what to do with the numbers.

A fasting insulin of 12 means something different in a 30-year-old marathon runner than in a 50-year-old sedentary executive. An ApoB of 95 means something different alongside an Lp(a) of 80 than alongside an Lp(a) of 20. An hsCRP of 2 in someone with chronic sinusitis means something different than in someone who just ran a marathon yesterday.

This is where a physician consultation pays for itself. We do not just order labs. We interpret them in your specific clinical context, build a personalized plan, and walk with you through implementation.

How LATOM works

STEP 1: BOOK A CONSULTATION

30-minute video visit. \$149. We review your symptoms, history, prior labs, and goals.

STEP 2: WE ORDER LABS

The 5 above, plus any others matched to your specific concerns. All through our partner laboratory network at wholesale pricing.

STEP 3: WE INTERPRET AND BUILD YOUR PLAN

Personalized supplement protocol through our Fullscript dispensary (15 to 30 percent off retail, practitioner-curated). Lifestyle interventions. Follow-up schedule.

STEP 4: WE FOLLOW UP

6-week check-in. Repeat labs as needed. Adjust the protocol based on response.

Ready?

Book your consultation at latomwellness.com.

Have questions first? Email info@latomwellness.com.

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